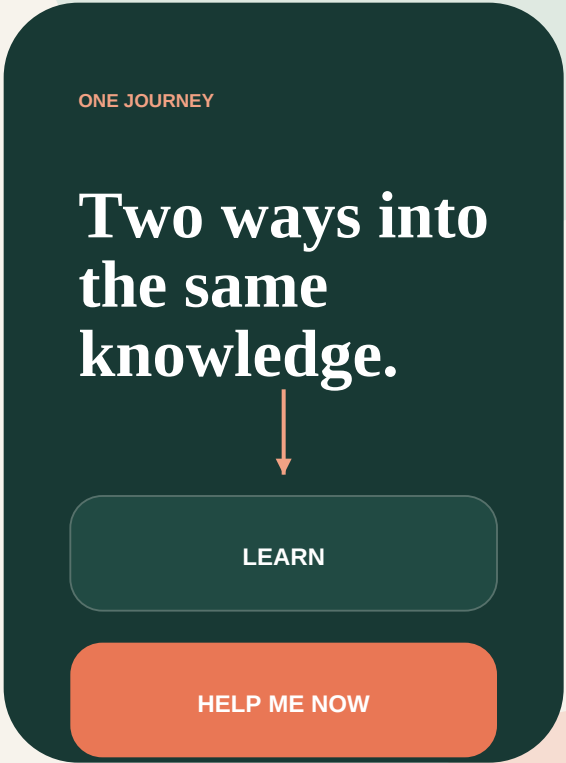


Caregiver Experience Content Wireframe

A scalable map of the user flow, knowledge hierarchy, and reusable content pieces required to build the experience.

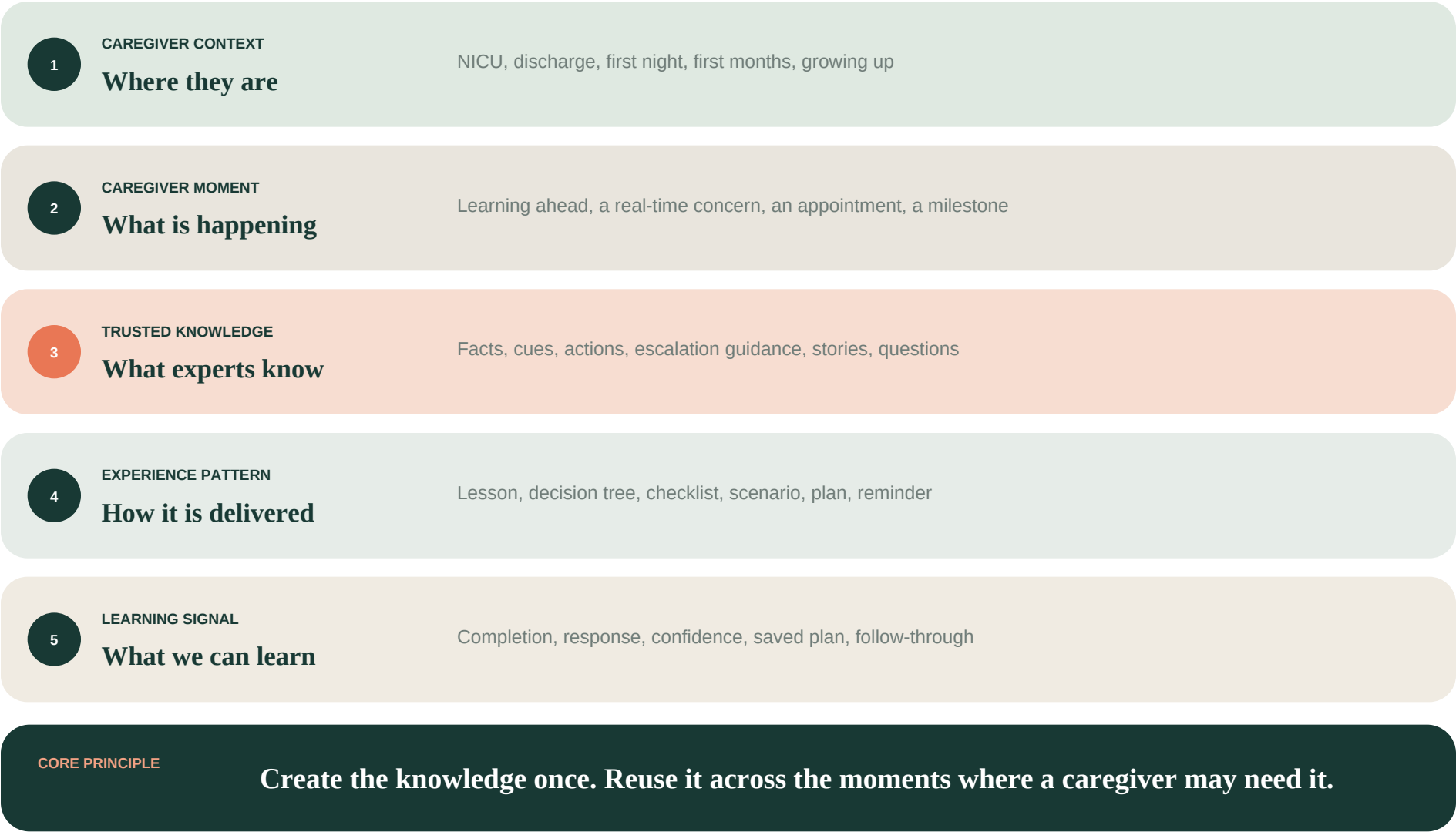


Built from trusted clinical and lived expertise



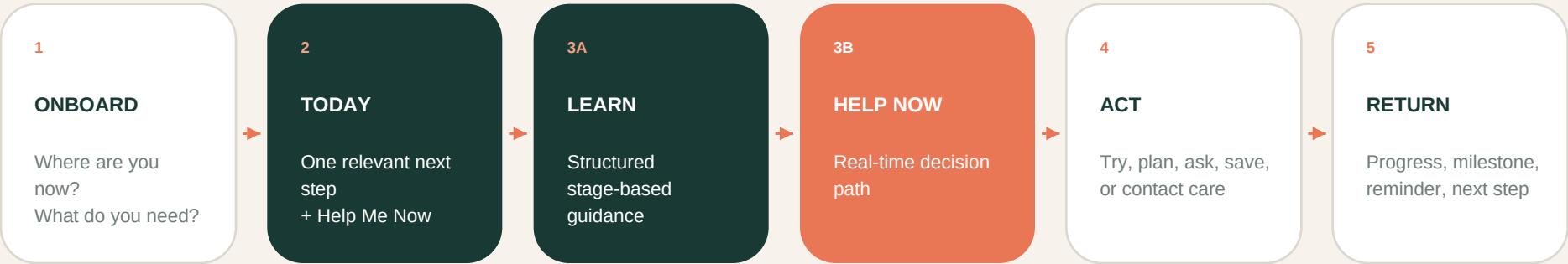
What the system is actually organizing

The platform is not a library of modules. It is a structured knowledge system that responds to a caregiver's stage and immediate need.

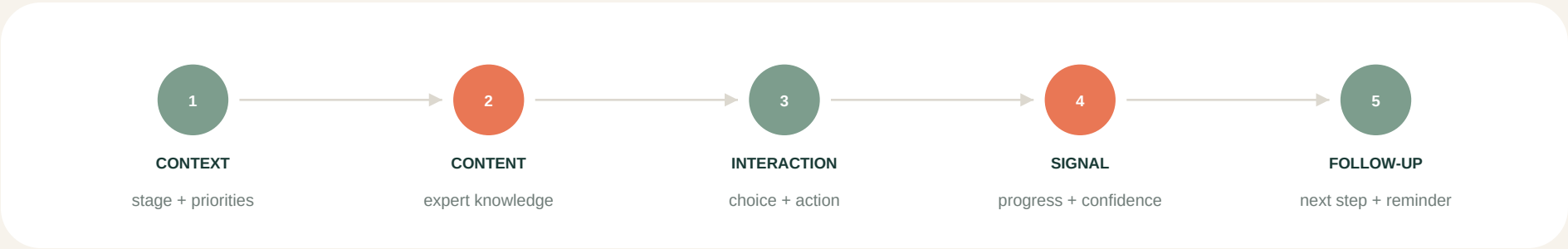


The caregiver experience as one continuous loop

Caregivers enter through a life stage or a need, take one useful action, and return to a journey that remembers where they are.



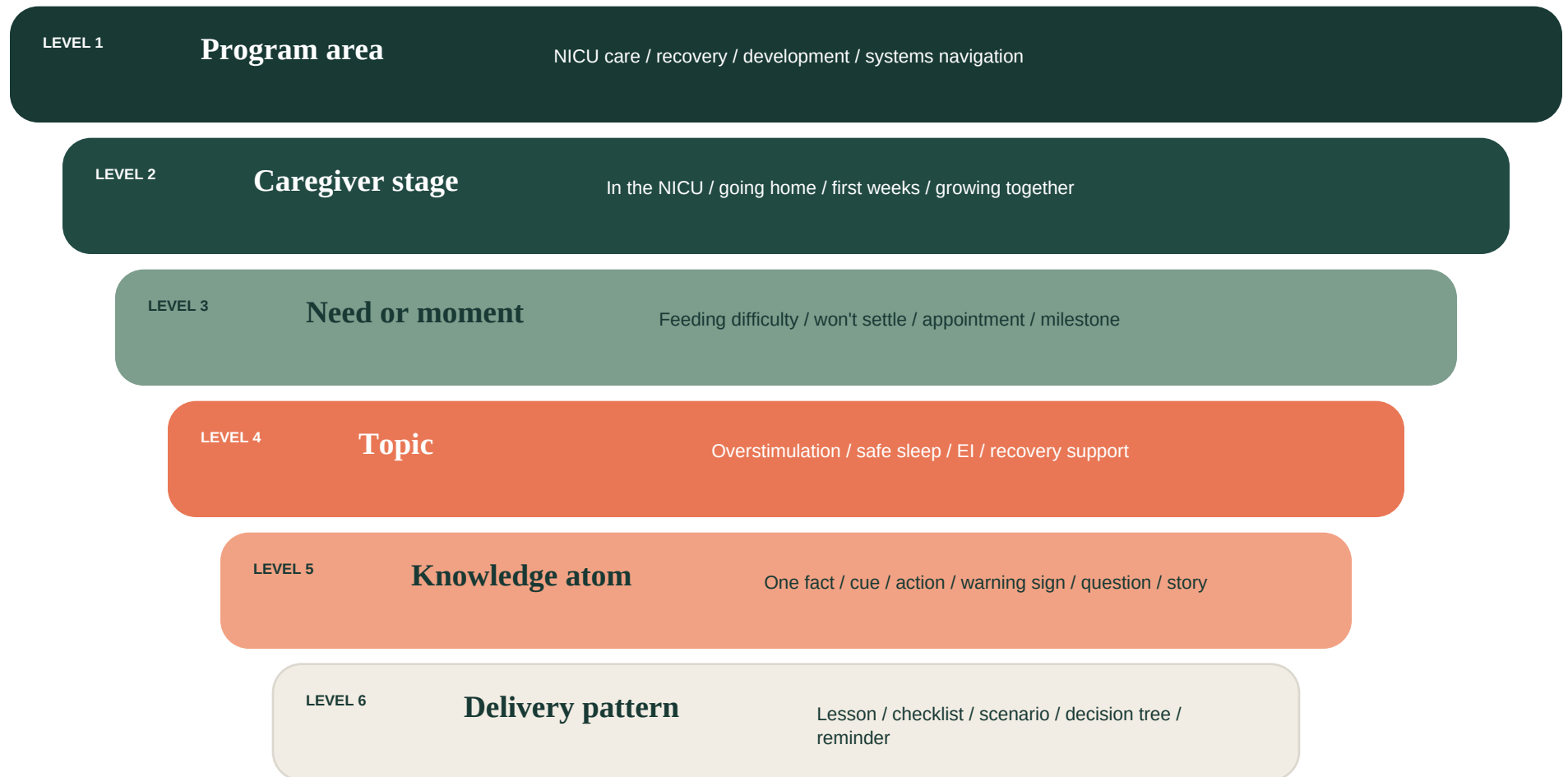
THE LOOP BENEATH THE SCREENS



Key idea: There is no dead end. Every content experience points to the next useful action, a care-team conversation, or the caregiver's larger journey.

The hierarchy: from program area to a single reusable atom

This is the backbone SMEs contribute to. Each layer adds context without duplicating the underlying knowledge.



SCALING RULE

Add new topics horizontally. Keep the six-layer structure stable.

The same content bundle repeats across topics

Each new topic becomes a column. The content requirements stay consistent, making gaps visible and production easier to plan.

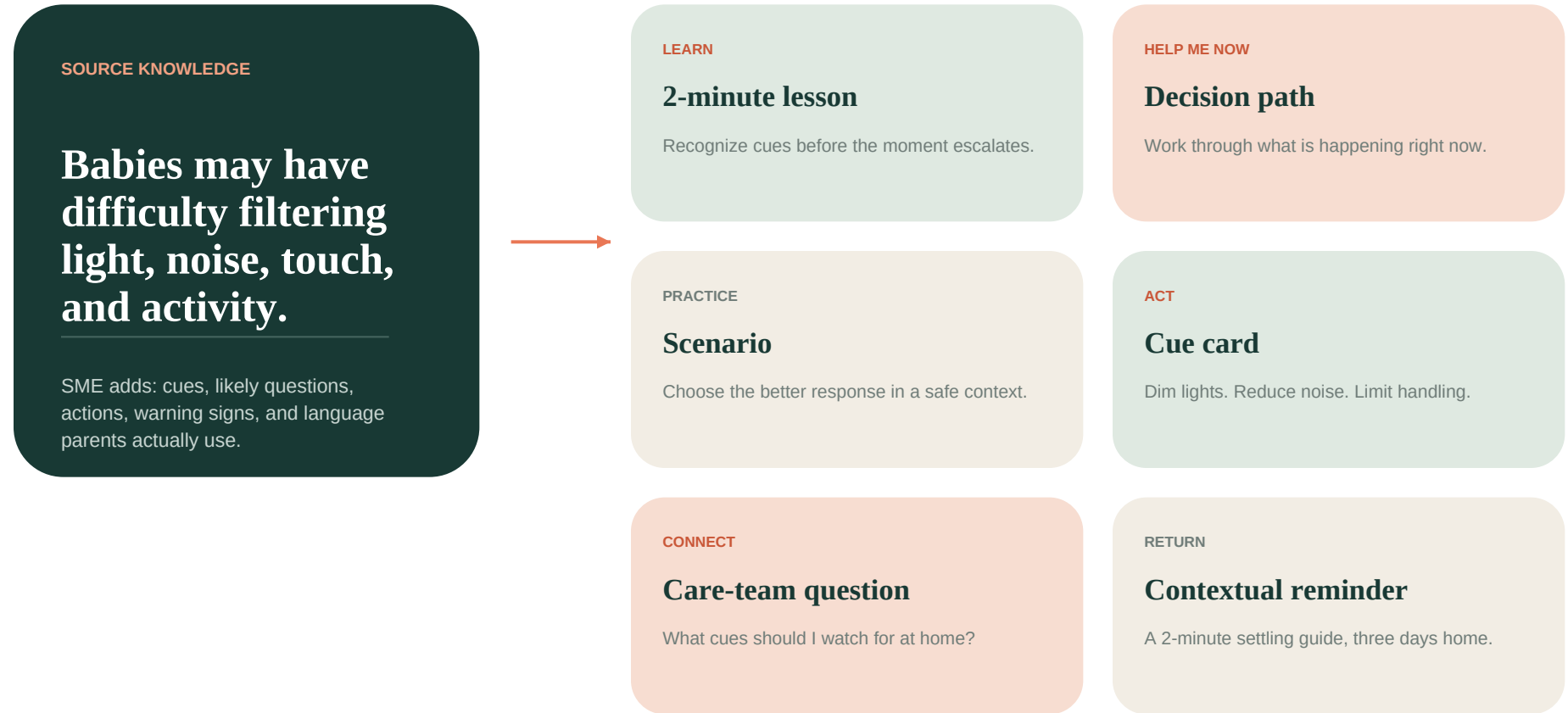
CONTENT BUNDLE	Over-stimulation	Feeding	Going home	Recovery support	Early Intervention	Behavior + regulation
Core explainer	✓	✓	✓	-	■	■
What to notice	✓	✓	-	■	■	-
What to do	✓	✓	✓	■	-	■
When to call	✓	✓	✓	-	■	■
Nurse perspective	✓	✓	-	■	■	-
Parent perspective	✓	-	■	■	-	■
Scenario / check	✓	■	✓	-	■	■
Question bank	✓	✓	✓	■	■	-
Reminder / nudge	✓	-	■	■	-	■

✓ CREATED ■ PLANNED - GAP TO DISCUSS

A matrix like this becomes the shared content roadmap.

One expert insight can power many experiences

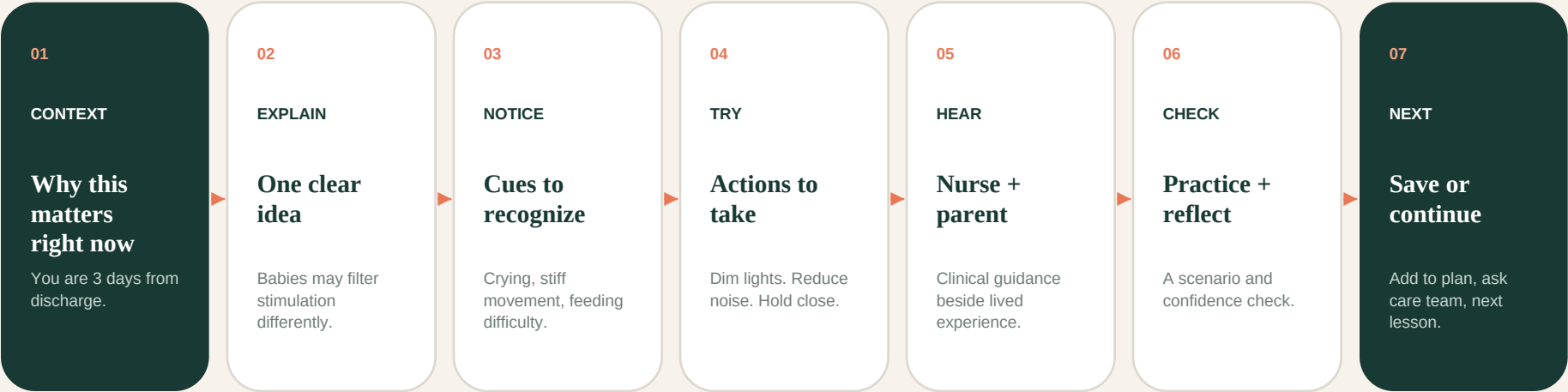
Example topic: understanding overstimulation. The source knowledge is reviewed once, then adapted to the caregiver's moment.



Reuse does not mean copy-paste. The fact stays stable; framing, length, interaction, and call-to-action change to fit the moment.

How structured learning moves through a topic

Every lesson is a short arc: relevance, recognition, action, human perspective, and one useful next step.



MINIMUM CONTENT NEEDED FOR EACH LEARN TOPIC



A lesson should answer: Why this matters, what I may notice, what I can try, and what I should do next.

How real-time guidance narrows the moment safely

This path supports observation and action. It does not diagnose, replace clinical judgment, or hide escalation guidance.



The pieces SMEs will help create

These are reusable building blocks. Not every topic needs every pattern, but the library makes the possibilities explicit.

KNOW

Explainer

One idea in plain language

What should a caregiver understand?

NOTICE

Cue card

Signs, behaviors, or changes

What might they notice?

DO

Action steps

A short sequence to try

What can they do safely?

PLAN

Checklist

Preparation or follow-through

What needs to be ready?

ASK

Question bank

Prompts for care conversations

What should they ask?

PRACTICE

Scenario

A realistic choice and feedback

What is the better response?

HEAR

Nurse tip

Clinical voice and nuance

What do you repeat every day?

RELATE

Parent story

Lived context and reassurance

What did experience teach?

REMEMBER

Reminder

Contextual follow-up later

When will this matter again?

ESCALATE

Safety guidance

Clear thresholds and contacts

When should they call?

REFLECT

Confidence check

Before / after self-report

Do they feel more prepared?

TRACK

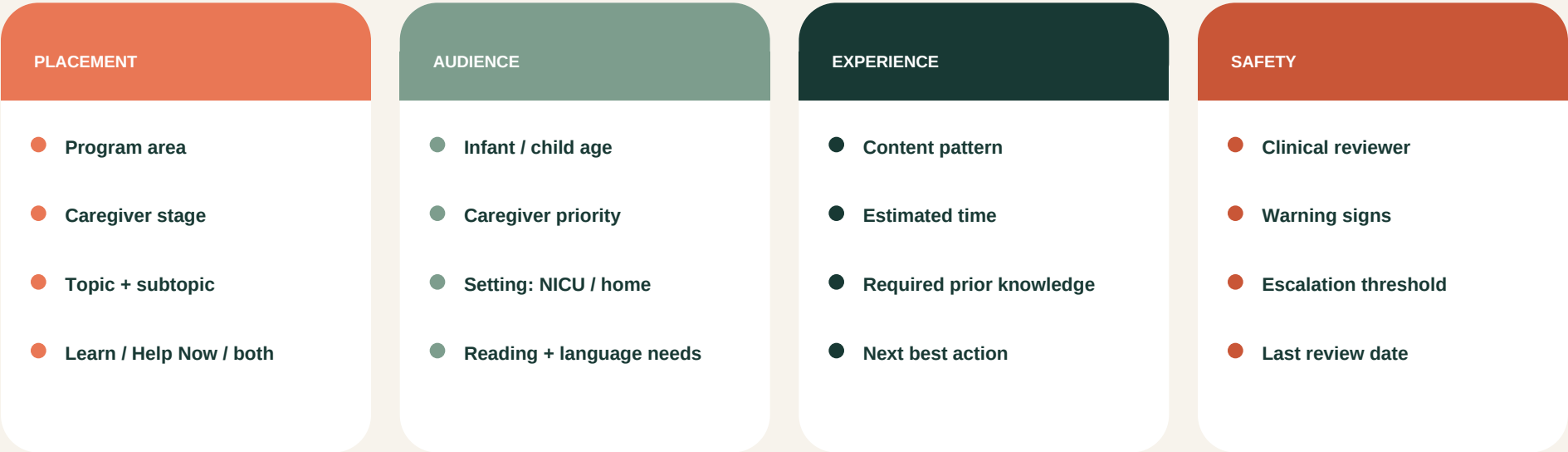
Milestone

A meaningful journey marker

What progress should feel visible?

Every content piece needs context around the content

Metadata is what allows the same knowledge to appear in the right place, at the right time, with the right safety controls.



EXAMPLE CONTENT RECORD

Overstimulation / Reduce stimulation / Action steps

NICU

FIRST WEEKS

LEARN + HELP NOW

2 MIN

CLINICAL REVIEWED

REVIEW: 6 MONTHS

WHY THIS MATTERS

Without metadata, content becomes a static library. With metadata, it becomes a responsive experience.

What we need to hear in a working session

The session starts with real moments, not screens. Answers are captured as content atoms and mapped into reusable patterns later.

N

NICU NURSE / CLINICAL SME

- 1 What do you explain over and over?
- 2 What do parents misunderstand?
- 3 What cues should they notice?
- 4 What can they try safely?
- 5 What should genuinely cause a call?
- 6 What questions matter before discharge?

P

PARENT / LIVED-EXPERIENCE SME

- 1 When did you think: why didn't anyone tell me?
- 2 What surprised or scared you?
- 3 What advice was not realistic?
- 4 What helped in the hard moments?
- 5 What did other parents understand?
- 6 What would you have wanted at 2 AM?

CAPTURE EACH ANSWER AS

FACT

CUE

ACTION

WARNING SIGN

QUESTION

STORY

SCENARIO

REMINDER

Start narrow enough to learn, complete enough to feel real

A strong first slice covers one caregiver stage, a small set of high-value moments, and the full range of reusable content patterns.

PILOT FOCUS

Preparing to Go Home

1 Comfort + regulation

2 Feeding

3 Warning signs + who to call

4 First night home

5 Recovery + support plan

Enough breadth to demonstrate Learn, Help Me Now, planning, care-team questions, and timely reminders.

SUGGESTED WORKSHOP SEQUENCE

1

Map moments

Which moments matter most from NICU to home?

2

Capture atoms

Facts, cues, actions, warning signs, questions, stories.

3

Choose patterns

Lesson, scenario, checklist, decision tree, reminder.

4

Review safety

Validate language, thresholds, contacts, and boundaries.

5

Prioritize slice

Select the smallest complete caregiver journey.

THE OUTCOME

A prioritized content map the SMEs recognize as true - and the product team can build from.